



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض رباعياً : نهاد على محمد منصور  
المحافظة : القاهرة الحي / المدينة : النزهه  
رقم البطاقة : 26012013400195 رقم التليفون : 01206077956  
العمر (تاريخ الميلاد) : 01-12-1960 المهنة : بدون عمل

### الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (.....) المقر بما فيه  
التوقيع: (.....)

التشخيص الإكلينيكي : اورام ليمفاوية  
نتائج الأبحاث : (مرفق)

التشخيص النهائي : اورام ليمفاوية  
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [6930] قسم العلاج : الاورام  
التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني  
الاسم : د/ندى حسن  
الاسم : د/نرمين رمضان  
الوظيفة : .....  
الوظيفة : .....  
الوظيفة : .....  
التوقيع : .....  
التوقيع : .....  
التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.  
تعتمد صحة الأبحاث والمرفقة  
رئيس اللجنة ومدير  
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

د. / ..... د. / ..... د. / ..... د. / .....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : .....  
جهة العلاج : .....  
ملاحظات : .....  
قسم : .....  
نفقات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

Name: نهاد علي محمد منصور  
National ID: 26012013400195



## تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of lymphoma for Further investigations and imaging

الوقت :

---:---

التاريخ :

05-07-2026

الطبيب المعالج :  
د/شيماء لاثمين





رسوم الاستعلام 20 جنية

تاريخ الإصدار: 2026-6-4

وجه الاستدراج برئاسة الهيئة

إلى من يهمه الأمر .....

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للتكافل التأميني لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاقات والأرامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 "الأطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية

وطبقا لقاعدة البيانات المرسلة من التأمين الاجتماعي

والبحث تبين أن



السيد (ة) نهاد علي محمد بطاقة رقم 26012013400195

تليفون رقم: 01288883276

وطبقا إلى خطاب من التأمينات الاجتماعية بغير عدم الالتحاق

بعض من الفئات غير المتكافئة بالتأمين الصحي صدر هذا الخطاب دون أدنى مسؤولية على الهيئة العامة للتأمين الصحي حال طسوع المواطن لقانون التأمين الصحي الشامل وشكرا

تاريخ الطباعة: 2026-6-4

الموقف  
شادي حسن العيسوي

الإدارة مركز تنظيم المعلومات  
بمكتب

مستقبل 1589474



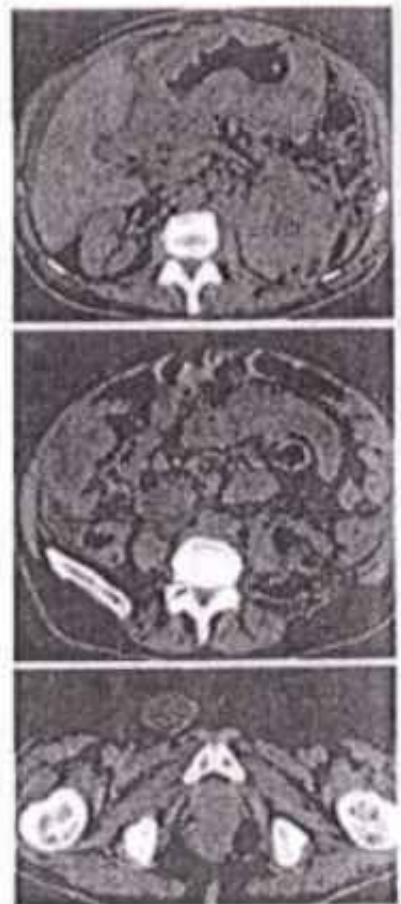
الخطاب سارو لمدة ثلاثة أشهر من تاريخ إصداره إلى حاله تغير هذه المواطن من قبل الهيئة العامة للتأمينات الاجتماعية أو وزارة التأمين الاجتماعي أو وزارة التجارة والتجارة

الإدارة المركزية لتنظيم المعلومات والتحول الرقمي

Name: Mr. Nehad Ali Mansour  
Ref.Prof.Dr.: Reda Fawzy Helmy  
Date: 27/04/2026

128 CT scan of the Abdomen & Pelvis was performed without contrast with special attention to the urinary system revealed:

- Significant diffuse circumferential polypoidal heterogeneous gastric mural thickening involving mainly the gastric fundus and body and to a lesser extent the gastric antrum reaching a maximum thickness of 3.7cm with no evidence of gastric outlet obstruction.
- The left kidney is enlarged in size with diffuse parenchymal edema associated with pronounced perinephric fat smudging and nodular thickening of the surrounding Zuckerkindl's and Gerota's fascia suggesting regional metastatic infiltration.
- Mild left renal pelvis fullness with diffuse circumferential upper ureteric mural urothelial thickening yet no obstructive lesions or stones.
- The liver is enlarged in size measuring about 16.2cm showing homogenous CT parenchyma associated with diffuse periportal edema.
- The gall bladder shows average capacity with few intraluminal stones or average size 5x5mm.
- Innumerable number of enlarged malignant looking pelviabdominal lymph nodes detected mainly at the gastric, celiac, peripancreatic, portahepatis, left perinephric, bilateral para-aortic, aortocaval, retrocaval and retrocrural groups, the largest seen at the gastric region measuring about 3x3cm.
- Mild to moderate pelviabdominal hyperdense ascites.
- Diffuse small intestinal mural thickening suggesting mural edema.
- Multifocal pelvic ileo-ileal short segment intussusception reaching about 4.5cm in length with no significant proximal intestinal dilatation.
- Pronounced diffuse mesenteric fat smudging.
- Right oblique inguinal hernia with fluid content.
- The lower chest cuts show bilateral pleural effusion, minimal on the right and mild on the left with left lower lobular posterior basal subsegmental subpleural pulmonary atelectasis.
- Multiple lingular subpleural pulmonary atelectatic bands with intervening ground glass haze.



*Name: Mr. Nehad Ali Mansour*  
*Ref.Prof.Dr.: Reda Fawzy Helmy*  
*Date: 27/04/2026*

- *The urinary bladder appears intact on the basis of non enhanced study. No stones within.*
- *Spleen, pancreas and suprarenal glands are of average size, regular margins with homogeneous attenuation pattern of the parenchyma. No focal mass lesion.*

*Opinion :*

- *Suspicious looking diffuse gastric mural thickening suggesting infiltrative malignant neoplastic process giving CT appearance of Linitis plastica.*
- *Bulky left kidney with evidence of parenchymal infiltrates and surrounding focal metastatic deposits.*
- *Gall bladder stones.*
- *Mild to moderate ascites.*
- *Innumerable number of suspicious looking periaabdominal lymph nodes mostly metastatic in nature.*
- *Multifocal distal ileo-ileal intussusception with no definite evidence of proximal intestinal dilatation.*
- *Right oblique inguinal hernia.*
- *Bilateral pleural effusion mainly on the left side.*

*Best regards*  
*Dr. Amira Mohamed, MD*

Dr.  
**Elia Anis Ishak**

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University  
Fellow, Armed Forces Institute of Pathology, USA  
Member, United States Canadian Academy of Pathology

Dr.  
**Shady Elia Anis**

M. Sc. MD Pathology, Asst. Prof. of Pathology, Cairo University  
Fellow, Mass General Hospital, Harvard, USA  
Member, United States Canadian Academy of Pathology

Patient Name : نهاد علي محمد منصور

Age : 65 Y : 5 M : 7 D

Gender : Male

Referred By Prof. Dr. : كبرليس عاطف

Clinical Diagnosis : Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb

Nature of Specimen : Block (Endoscopic biopsy, labelled as 1053620)

Receiving Date : 2020-05-08

Analysis ID : 1055769

Conf. Code : 0600

Patient ID : 771992

## IMMUNOHISTOCHEMISTRY REPORT

### Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- CD 20 \ CD 3 \ Ki67 \ CD 10 \ C-MYC \ Bcl 2 \ Bcl 6 \ MUM 1.

### Results :

Neoplastic lymphoid cells are POSITIVE for CD 20.

Neoplastic lymphoid cells are NEGATIVE for CD 3.

90 % of neoplastic lymphoid are POSITIVE for Ki67.

Neoplastic lymphoid cells are NEGATIVE for CD10.

40% of neoplastic lymphoid cells are POSITIVE for C-MYC.

Neoplastic lymphoid cells are POSITIVE for Bcl 2.

Neoplastic lymphoid cells are POSITIVE for Bcl 6.

Neoplastic lymphoid cells are POSITIVE for MUM 1.

### Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH LARGE B-CELL LYMPHOMA OF POST GERMINAL CENTER ORIGIN (ACTIVATED B-CELL TYPE), TRIPLE EXPRESSOR.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

*Shady Elia*

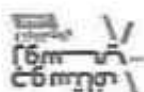
*Elia Anis*



This report is electronically signed, any abrasion or modification would render it void.

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع  
تيلفون : ٠١٩١٢٩٤٥١٤ - ٠١٢٨٧٠٠٥٢١٧



Nihad Ali Mohammed

2026-06-01

Prof.Dr. no doctor

Goustave Roussey International Egypt

Patient's Name:

Date:

Referred by:

Hospital:

## NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH

### CORONAL RECONSTRUCTION: -

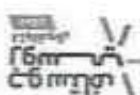
#### Findings:

- Bilateral moderate pleural effusion with underlying consolidation collapse.
- Bilateral upper lung lobes extensive ground glassing mounting to consolidations with mild bronchiectatic changes.
- Bilateral upper lung lobar multiple thick atelectatic/fibrotic bands.
- No hilar or mediastinal lymph nodes.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.

#### OPINION:

- Bilateral moderate pleural effusion with underlying consolidation collapse.
- Bilateral upper lung lobes extensive ground glassing mounting to pneumonic consolidations with mild bronchiectatic changes.
- Bilateral upper lung lobar multiple thick atelectatic/fibrotic bands.

Dr. Mai Shaaban MD



Nihad Ali Mohammed

2026-06-01

Prof.Dr. no doctor

Goustave Roussey International Egypt

Patient's Name:

Date:

Referred by:

Hospital:

## NON-CONTRAST MULTI-SLICE CT SCAN OF THE PARA NASAL SINUSES IN THE CORONAL PLANE WITH SAGITTAL RECONSTRUCTION:

### Findings:

- Mucosal thickening seen involving both maxillary antra with obliteration of the osteometal complex on both sides.
- Mucosal thickening is also seen partially opacifying the ethmoidal air cells on both sides.
- Mucosal thickening of both sphenoidal and left frontal sinuses.
- Clear other scanned paranasal sinuses with intact bony boundaries.
- No air/fluid levels are seen.
- The nasal septum is not significantly deviated.
- Clear nasopharynx.

### OPINION:

- Findings are those of bilateral maxillary and ethmoidal chronic sinusitis.
- Mucosal thickening of both sphenoidal and left frontal sinuses.

Dr. Mai Shaaban MD

*This report is electronically signed, any abrasion or modification would render it void.*

Shahid

**Prof. Dr. Shady Elia Anis**

*Elia Amir*

**Prof. Dr. Elia Anis Ishak**

Block, findings are compatible with large B-cell lymphoma of post germinal center origin (activated B-cell type), triple expressor.

**Conclusion:**

Neoplastic lymphoid cells are POSITIVE for CD 20.  
Neoplastic lymphoid cells are NEGATIVE for CD 3.  
90 % of neoplastic lymphoid are POSITIVE for Ki67.  
Neoplastic lymphoid cells are NEGATIVE for CD10.  
40% of neoplastic lymphoid cells are POSITIVE for C-MYC.  
Neoplastic lymphoid cells are POSITIVE for Bcl 2.  
Neoplastic lymphoid cells are POSITIVE for Bcl 6.  
Neoplastic lymphoid cells are POSITIVE for MUM 1.

### Results:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: - CD 20 \ CD 3 \ Ki67 \ CD 10 \ IC-MYC \ Bcl 2 \ Bcl 6 \ MUM 1.

Technique:

# IMMUNOHISTOCHEMISTRY REPORT

Receiving Date : 2026-05-08  
Patient ID : 771992

Nature of Specimen : Block (Endoscopic biopsy, labelled as 1053620)

**Clinical Diagnosis :** Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb

Referred By Prof. Dr. : **عبدالله بن عيسى**

Age : 65 Y : 5 M : 7 D  
Gender : Male

Conf. Code : 0600

Patient Name : \_\_\_\_\_

Analysis ID :  1055/69

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University  
Fellow, Armed Forces Institute of Pathology, USA  
Member, United States Canadian Academy of Pathology

Elia Anis Ishak

DR.

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University  
Fellow, Mass General Hospital, Harvard, USA  
Member, United States Canadian Academy of Pathology

**Shady Elia Anis**

Dr.



علاوة رئيس - بلاك أ - ميدان رئيس - اللور السلس و السلس  
تليفون : ١٢١١٩٤٥١١٥ - ٠١٢٨٧٠٠٥١٧

1/1

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Prof. Dr. Elia Anis Ishak

Prof. Dr. Shady Elia Anis

**Diagnosis :**  
Gastric ulcer, Endoscopic biopsy, MALIGNANT ROUND CELL TUMOR, LIKELY LYMPHOMATOUS.  
- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION AND SUBCLASSIFICATION.

Sections examined from the specimen revealed pieces of gastric mucosa with focal ulceration overlying dense diffuse infiltration by large and intermediate sized round cells with large nuclei and prominent nucleoli with moderate mitotic activity. There are small curved bacilli related to surface epithelium.

**Gross :**  
Multiple tan tissue pieces, collectively measured 0.7x0.7 cm, totally submitted.

## PATHOLOGY REPORT

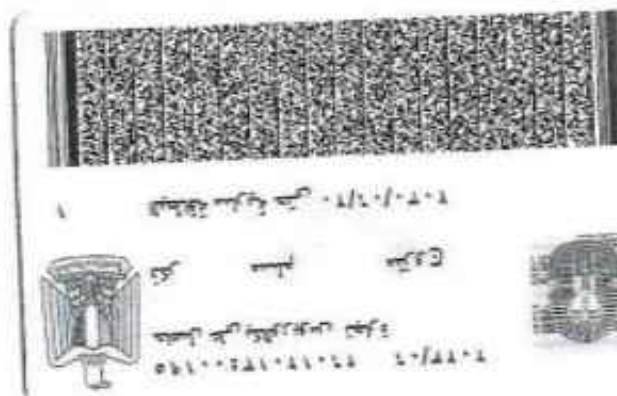
**Patient Name :** علي محمد منصور  
**Age :** 65 Y : 5 M : 1 D  
**Gender :** Male  
**Referred By** Prof. Dr. : علي عيسى  
**Clinical Diagnosis :** Large malignant ulcer invading gastric antral mucosa and extending to duodenal bulb  
**Nature of Specimen :** Endoscopic biopsy  
**Receiving Date :** 2026-05-02  
**Patient ID :** 771992

**Analysis ID :** 1053620

**Conf. Code :** 1990

**Dr. Shady Elia Anis**  
M.Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University  
Fellow, Mass General Hospital, Harvard, USA  
Member, United States Canadian Academy of Pathology

**Dr. Elia Anis Ishak**  
M.Sc. Ph.D. Pathology - Prof. of Pathology, Cairo University  
Fellow, Armed Forces Institute of Pathology, USA  
Member, United States Canadian Academy of Pathology





S C A N

Name: Nehad Ali Mohamed Mansour  
Referred by: Prof. Kerollos Atef Gaber

ID: 5154475  
Date: 04/05/2026

### <sup>18</sup>F-FDG PET/CT examination

#### Clinical Indication:

- Patient with history of surgically treated left breast cancer followed by chemoradiotherapy ended in 2019.
- Currently presented with suspected gastric cancer.

Administered activity: 244.2 MBq. <sup>18</sup>F-FDG

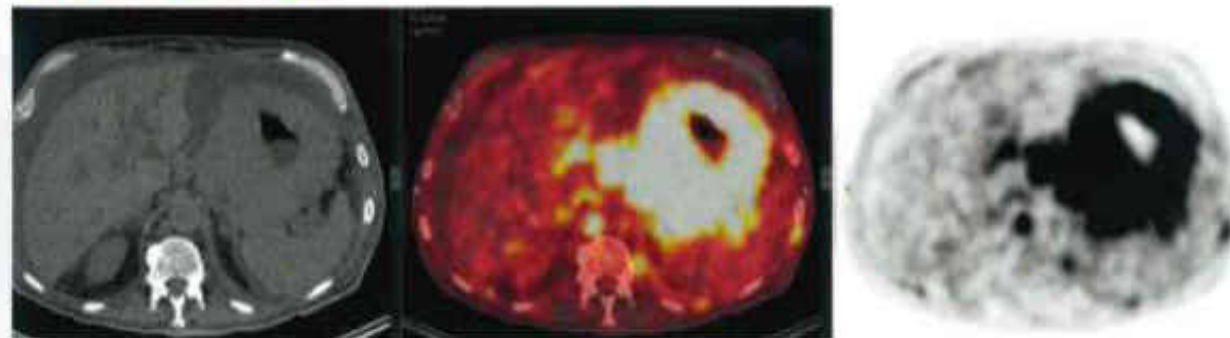
B.G.L. : 75 mg/dL

#### Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Non-Contrast multislice CT examination of the same region.

In reference to the previous FDG PET/CT report dated 02/03/2022 (no DICOM images were available at time of interpretation)

#### I. PET/CT Findings:



- **Stomach:** increased FDG uptake corresponding to mural thickening involving the gastric fundus, body and antrum, SUVmax 11.8 as measured over 5.5 cm thickness in the body.
- **Lymph nodes:** increased FDG uptake by:
  - **Cervical:** bilateral levels II, III and right IV nodes, SUVmax 6.8 as measured over 1.8 cm left level II node.

الفاسكان

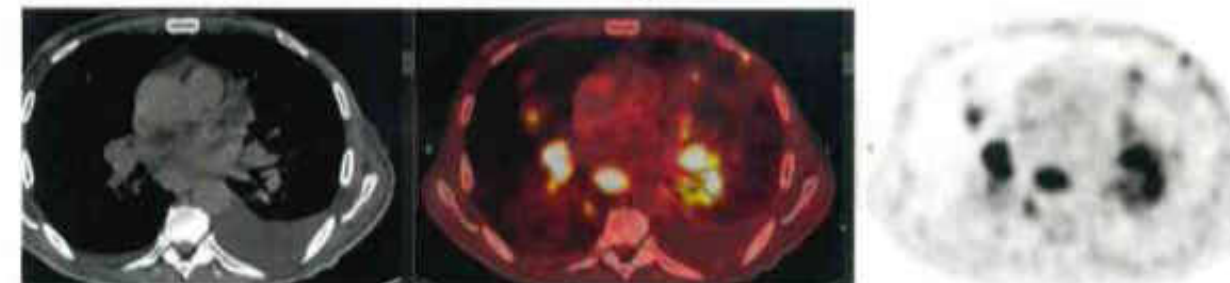
16171



S C A N



- **Mediastinal:** bilateral internal mammary, superior mediastinal, pre-vascular, pre-tracheal, bilateral hilar, aortopulmonary, sub-carinal, bilateral retrocrural and epiphrenic nodes, SUVmax 8.5 as measured over 2.7x 2 cm right hilar LN.



- **Abdomino-pelvic:** perigastric, celiac, porta-hepatis, porto-caval, aorto-caval, left para-aortic, mesenteric, bilateral common and left external iliac nodes, SUVmax 14 as measured over 3.9x 2.3 cm porto-caval LN.

(Newly developed in reference to the previous PET/CT report)



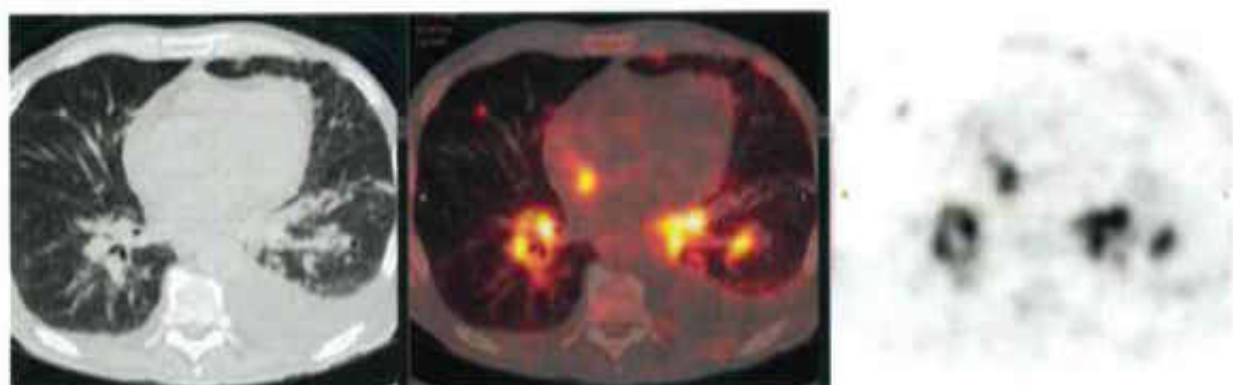
- **Peritoneum:**

الفاسكان

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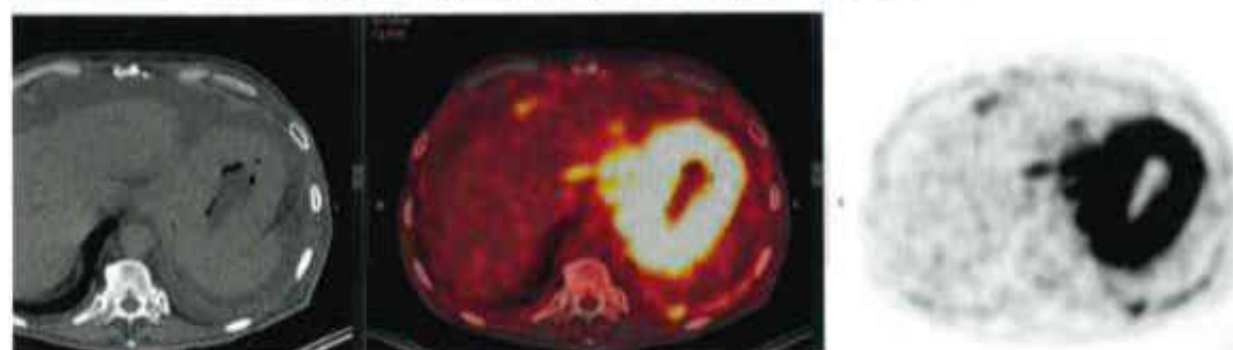
- increased FDG uptake corresponding to sheet like thickening seen perigastric, perihepatic (SUVmax 3.3), perisplenic, underneath anterior abdominal wall, para-colic, along pelvic reflections and along right inguinal hernia, the latter with SUVmax 4.7. (newly developed in reference to the previous PET/CT report).

- Moderate ascites.



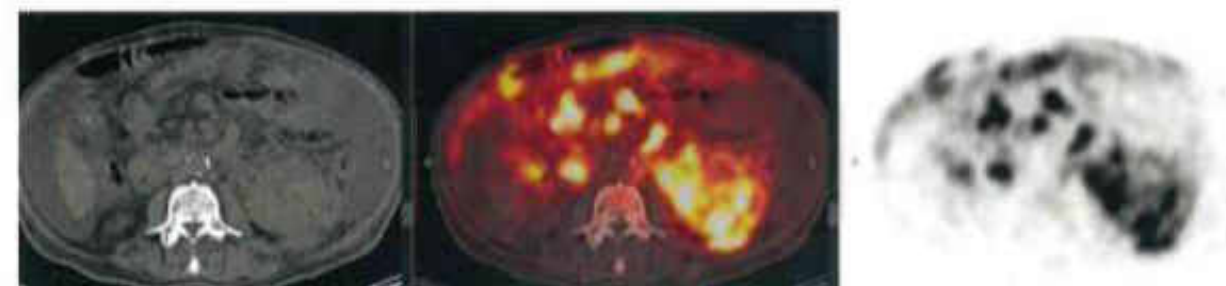
#### • Lungs:

- FDG avid bilateral multiple pulmonary nodules and nodular densities, SUVmax 2.5 as measured over 5 mm in the middle lobe. (newly developed in reference to the previous PET/CT report).
- Bilateral atelectatic changes and ground glass densities.



#### • Pleurae:

- Increased FDG uptake corresponding to mild sheet like thickening involving bilateral costal pleurae and along pleural recess, SUVmax 3.7 as measured over left posterior costal pleura. (newly developed in reference to the previous PET/CT report).
- Bilateral mild pleural effusion more on the left side associated with relaxation atelectasis.



- **Left kidney:** increased FDG uptake within ill-defined lesion infiltrating the whole parenchyma, SUVmax 7.9. (newly developed in reference to the previous PET/CT report).



- **Small bowel:** multiple areas of mural thickening are seen involving jejunal and ileal loops with increased FDG uptake, SUVmax 14.3 within ileal loop in the left iliac fossa.
- **Right kidney:** no FDG avid lesions.
- **Liver:** average sized with no FDG avid lesions. No dilated biliary radicles.



FDG uptake within bilateral nodular adrenal thickening,  
0 on the right side, for follow up.

DW:

OG avid lesions.

enerative changes of the spine.

DG avid lesions.

ctomy bed and axillary dissection bed: no  
sions.

st: no FDG avid lesions.

3 uptake within enlarged thyroid gland, *for  
and sonographic correlation.*

#### CT Findings:

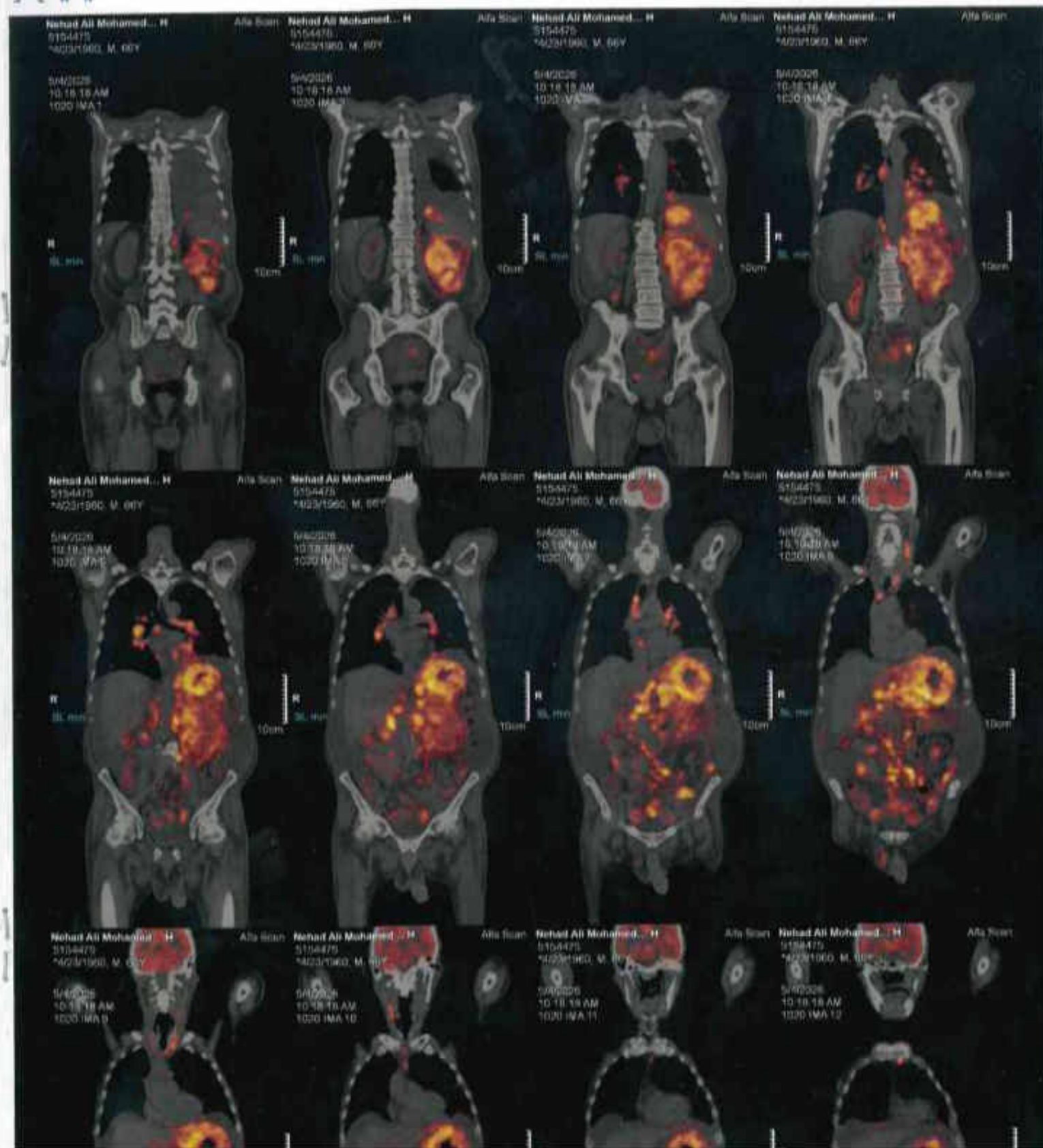
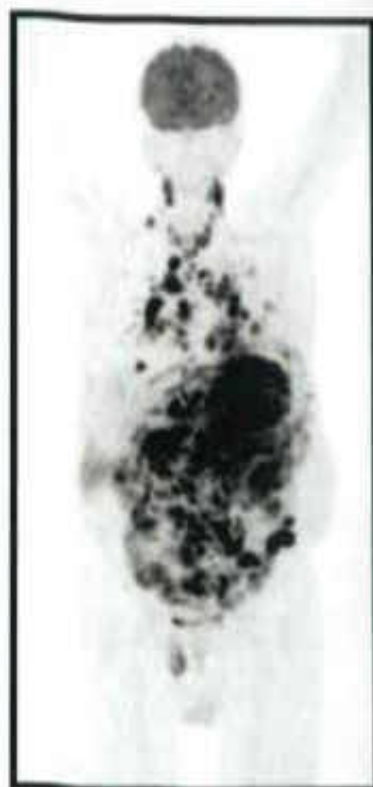
, pancreas, pelvic organs, aorta and IVC  
markable.

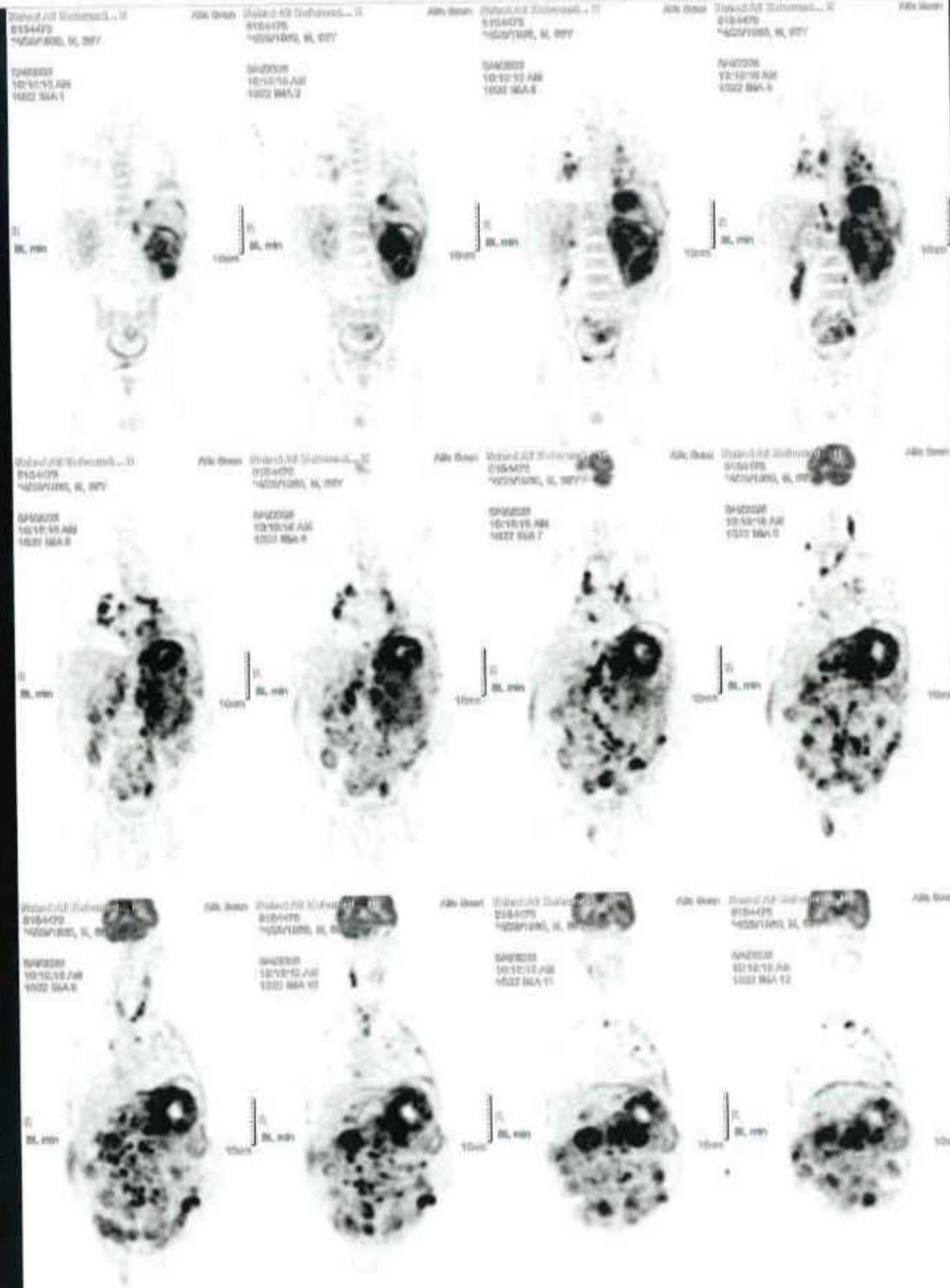
#### rdial effusion.

ily active gastric, lymph nodal, peritoneal, pulmonary,  
all bowel and left renal neoplastic lesions as described.  
is among diagnostic possibilities, for histopathology.

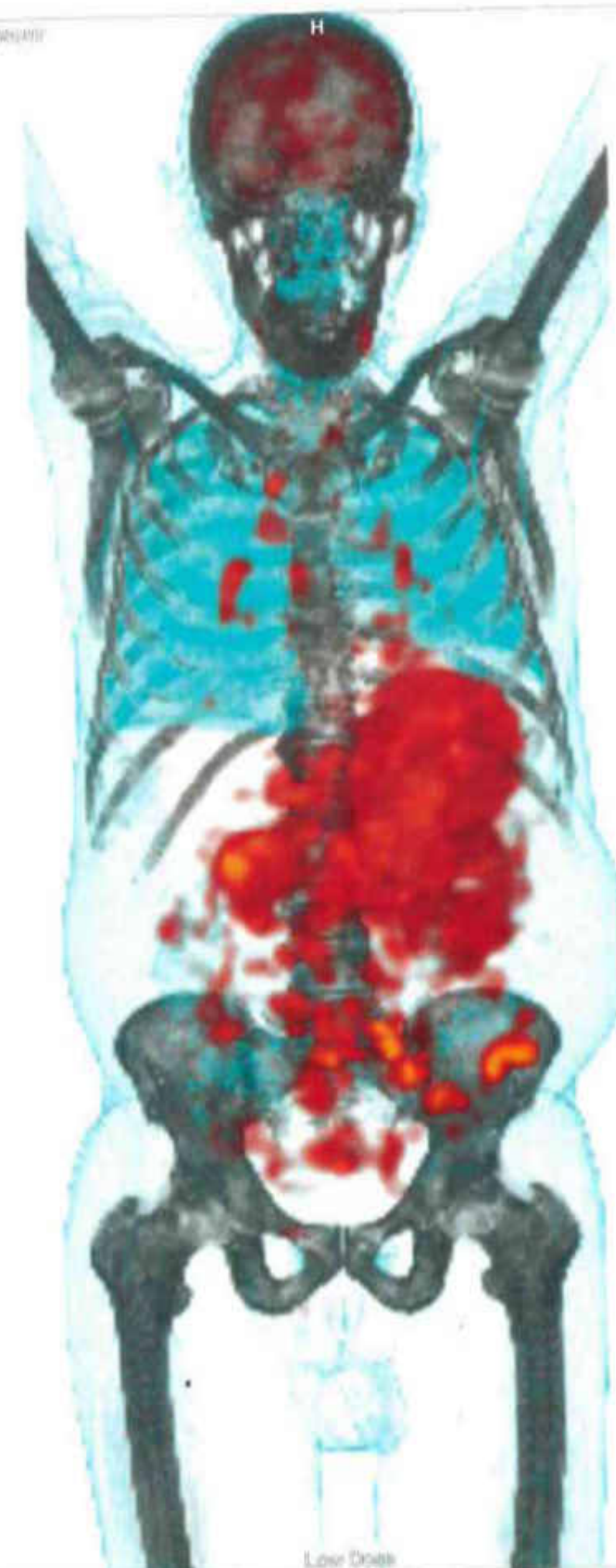
st  
seny, MD

**Nuclear Medicine Physicians**  
**Dr. Sherif El-Refaei, MD, EBNM**  
**Dr. Maha Khalil, MD**





Patient ID: 0154475  
 Name: HOSSEIN ALI GADAR  
 Date: 10/05/2020  
 Time: 10:17:41 AM  
 Location: 1002 SGA 1



AG 1  
 CT\_BONE SPI

Zoom 10.43  
 630\*1254



إسم المريض ثلاثي: رضا رضا رضا  
رقم الملف: .....

تقرير طبي

الميد:

تحية طيبة وبعد،،

إيماء الطالب المقدم من

بقسم: رقم طبي

تنتعرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول: تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد أنه:

المريض يعاني من آلام بالصدر اليساري (DLCL)  
ويعتبر رجوعه إلى المستشفى والحمد لله

Anacolin  
Atrialiumain  
(CHOP) Endoxin  
std  
بإجراء فحص الدم

مع إجراء فحص الدم مع ملاحظة أن المريض يعاني من آلام بالصدر اليساري والرجوع إلى المستشفى

تحريراً في

التاريخ: ٢٠١٩/٩/٢٠

الوقت: .....

الطبيب المعالج  
الطبيب العام

يؤكد

اسم المريض ثلاثي: زيار علي محمد  
رقم الملف: .....

## تقرير طبي

الميد:

تحية طيبة وبعد،،

إيماء للطلاب المقدم من .....

بقسم: رقم طبي: .....

نشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول: تاريخ وساعة الخروج: .....

وبعد الكشف والفحوصات والمتابعة وجد انه:

المرضى يعاني من مرض بالسرطان الليناري (DLBCL

(CD20 +ve)

والمرضى يعاني من لا يستطاع ان يمشي بها الى السرير (Rudimentary) ما قبل

بفرقة: 7- الخمسة عشر من كل مائة استبان مع لدرج في مريض

وذلك لظهور استجابة جيدة له بعد العلاج

PET CT 12/9، و

(Marked Regression Course)

تحريراً في

التاريخ: ٢٠١٩/٩-٢٠٢٠

الوقت: .....

الطبيب المعالج

د. محمد علي محمد

بمقتضى: ..

|                                             |                              |
|---------------------------------------------|------------------------------|
| Name : Nihad Ali Mohammed Mansour           | Date : 12-09-2026 07:03      |
| Branch : Nasr City                          | Medical No. : 459430         |
| Ref. Physician Prof : Goustav Roussey Egypt | Ins. Company : التأمين الصحي |

## **Follow-up Whole body (FDG) PET/CT Study:**

### **Clinical history:**

- 65-year-old male patient with a history of surgically managed left breast cancer, with recent biopsy-proven large B-cell lymphoma from large gastric ulcerative lesion.
- He received chemotherapy till 30/08/2026
- PET/CT is requested for status evaluation and therapy monitoring.

### **Technique:**

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 2.5**.

### **PET/CT findings:**

Considering the previous PET/CT report dated: 04/05/2026.

### **Head and neck:**

- Metabolic and morphologic regression of the prior reported multiple enlarged bilateral upper/lower deep cervical currently few, reaching (1 cm compared to 1.8 cm) and eliciting SUVmax up to 2.6 compared to 6.8.
- No other pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.
- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.

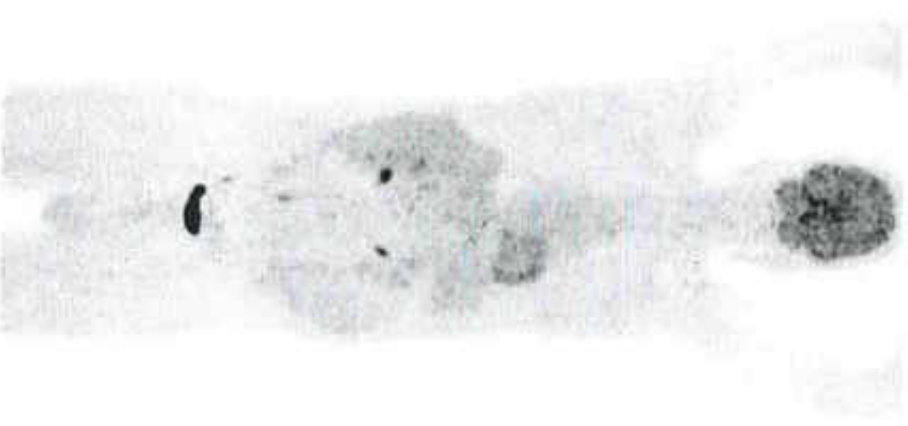
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### Chest:

- Clear left mastectomy bed.
- Normal right breast appearance.
- Almost resolution of the prior reported mediastinal and bilateral hilar lymph nodes.
- No current definite lung nodules or parenchymal infiltrates.
- No current pleural thickening, masses or effusion.

### Abdomen and Pelvis:

- Metabolic and morphologic regression of the prior reported FDG-avid diffuse gastric mural thickening currently up to 3.2 compared to 5.5 cm in max thickness and eliciting SUVmax up to 2.6 compared to 11.8.
- Metabolic regression of the prior reported left renal ill-defined infiltrating mass currently eliciting SUVmax up to 2.9 compared to 7.9.
- Almost resolution of the prior reported abdomino-pelvic lymph nodes currently sub-cm with no significant metabolic activity.
- Metabolic remission of the prior reported bilateral nodular adrenal thickening.
- Almost resolution of the prior reported omento-peritoneal nodular thickening.
- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis), tracer-avid foci or biliary radicles' dilatation could be detected.
- Unremarkable spleen, right kidney, pancreas and pelvic organs.
- No current evidence of ascites, free or encysted abdomino-pelvic fluid collections.



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### Musculoskeletal & BM:

- No any pathological sclerotic/lytic bony lesions or abnormal tracer-avid foci could be detected over the surveyed skeleton.

### Impression:

Follow-up whole-body (FDG) PET/CT study shows:

- Maintained Clear left mastectomy bed.
- Metabolic and morphologic regression of the prior reported FDG-avid diffuse gastric mural thickening.
- Metabolic and morphologic regression of the prior reported multiple enlarged cervical lymph nodes, currantly coping with Deauville score III.
- Metabolic regression of the prior reported left renal ill-defined infiltrating mass.
- Almost resolution of the prior reported mediastinal bilateral hilar, abdominal, pelvic lymph nodes as well as peritoneal, pulmonary, pleural and small bowel infiltrates.
- No newly developed nodal or extra nodal metabolically active lesions.

Best regards,

Dr. Youssef Ragab Gad-Allah, MSc.

Dr. Alaa Nagi, MD.  
Nuclear Medicine



